OCD-08 | For therapists | OCD, ages 8 and over

Maintaining learning and planning next steps

1 | Purpose, fit and boundaries

Purpose of this unit

Summarize meaningful learning and functioning, identify changes warranting contact and update the plan with the therapist. This promises neither prevention of recurrence nor instructions for restarting treatment alone.

Choice and adaptation

Choose this sheet following professional assessment and with consent. The person may skip questions, answer aloud, or choose another tool. This is proposed original material for review; it provides no score, diagnosis, or promise of an outcome.

Boundaries specific to this unit

Returning doubt is not necessarily failure. Do not constantly monitor to ensure no signs remain. Significant functional or other changes require assessment, not automatically restarting an old exercise or changing medication.

Reusing existing material

Unit 8 gathers decisions from units 2–7. General session summaries stay in SES; this sheet records learning about the OCD cycle and specific contact criteria agreed in treatment.

A fictional example for discussion

Fictional example: After a helpful period, Michal notices new demands prolonging checks. She brings the change to her session as agreed, without proving every thought disappeared or deriving practice dosage from the sheet.

> Draft for administrator review. Clinical review and final rights approval are still pending.

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Working together with the worksheet

2 | A conversation about one situation

Opening

Choose a situation the person wants to explore. It need not be the hardest situation. Check that the question is understood and the wording fits the person; their account comes before the therapist's interpretation.

1 | Question for discussion

What functional goals advanced and what remain open?

2 | Question for discussion

What main learning do I want to take from treatment?

3 | Question for discussion

What defined conditions call for renewed contact or assessment?

4 | Question for discussion

What was agreed about monitoring, support and ongoing treatment?

5 | Question for discussion

What will I update with the therapist if life circumstances change?

Closing

Summarize what became clearer and what remains open. Use the existing SES-S01 and SES-S02 for the session framework. There is no need to repeat an assessment or plan already recorded.

Possible difficulties

Long sign lists may encourage monitoring. Choose a brief, functionally meaningful description. If ending raises insecurity, explore support and follow-up without promising unagreed availability or outcomes.

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Adapting for four age groups

3 | Age is a starting point, not an assumption about ability

CH | Children aged 8–11 only

Ages 8–11 only. Read one question and allow brief or spoken answers. Parents add separate observations without deciding the child's thoughts; do not test reporting accuracy. Parents record functional change without repeatedly asking about thoughts.

Offer one question at a time with little writing. Adult support must not replace the child's own voice.

TE | Adolescents, approximately ages 12 to 17

Ages 12–17. Agree what stays private and what is shared with family or school, subject to safeguarding duties. Embarrassing thought content need not be disclosed. Consider transitions between settings in follow-up planning.

Agree in advance what will be shared and with whom, and explain the privacy limits in this setting. Do not promise absolute confidentiality.

AD | Adults

Connect work to chosen goals, family, employment and existing care. Distinguish rituals, practical needs and value-consistent behavior. Medication changes remain with the appropriate treating professional.

OA | Older adults

Preserve adult language, choice and privacy. Assess access, health and desired support without automatically attributing new changes to OCD or assuming reduced ability. New medical assessment is separate from assuming OCD recurrence.

Offer reading support, time, and conversation according to preference. Choose help from another person together with the participant, respecting their privacy.

> Each age version needs its own review. Approval of one version does not automatically approve the others.

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Practice, review and next steps

4 | Shared choice and learning from the attempt

Before offering practice sheet P

The P sheet proposes recording decisions and learning within treatment, not ERP instructions. The therapist determines suitability, action, scope and support. Without an agreed plan, use only for discussion. Do not begin exposure or change response prevention based on this sheet.

At the next session

Review functioning, wishes and new conditions. Together decide whether assessment, continued support or a plan change is needed; a unit number does not determine treatment ending.

Professional and technical records

Each version has a separate review record covering age suitability, originality, rights, and fictional filling. Reviewer and date fields remain blank until assigned. File checks or a code merge are not clinical approval. Clinical information stays only in the authorized setting, outside the code repository.

Background references

The following links were used for background only. Questions and adaptations were written afresh for the expansion plan; no questionnaire, scale, or protocol was copied. Adult sources do not validate the child versions. This material makes no claim about session count or treatment effectiveness.

Treatment-manual mapping status

Mapping against a full OCD manual and professional approval remain pending. This is original support material for review, not an approved ERP manual or its replacement.

www.nice.org.uk/guidance/cg31/chapter/Recommendations
iocdf.org/about-ocd/ocd-treatment-guide/erp/
kids.iocdf.org/professionals/md/treatment/

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